

ENGLISH FOR WORK / SEPTEMBER 2026

Home Care & Caregiving English



Teacher's guide

Teach practical workplace communication with specific cases, clear language targets, and a repeatable feedback process.

PRACTICE THAT TRANSFERS TO WORK

Read a realistic case. Study complete conversations. Find the right language, speak, get feedback, and try again.

Eight lessons | Intermediate to advanced | Independent or classroom study

For Home care aides, personal care assistants, and caregivers.

Fictional workplace-language practice, not clinical training. Duties vary by qualification, care plan, employer, and location. Obtain consent, follow approved procedures, protect private information, and refer clinical or emergency decisions to authorized professionals.

All scenarios, figures, and model responses are fictional teaching material. Use invented details in practice.

[Open this course on English Ladder](#)

Your course at a glance

Use the lesson numbers to match this document with the website and the other guides.

01 Confirming the visit and the care plan

Name the mismatch between the assignment and the client's expectation without blaming either person.

02 Consent, choice, and respectful assistance

Make consent and preferences explicit while explaining a real time limit.

03 Reporting a change without diagnosing

Report a change using a source, time, and clear limit on your knowledge.

04 Boundaries around medicines and clinical questions

Respond helpfully to a request that exceeds the caregiver's authorized role.

05 Meals, preferences, and unresolved restrictions

Separate an available approved option from a requested but unverified substitution.

06 Privacy and family communication

Explain an identity or permission check and provide a legitimate route for the caller's request.

07 A handoff the next person can use

Separate completed work, unresolved work, and responsibility for the next step.

08 Delays, complaints, and reliable follow-up

Acknowledge the disruption, label an estimate, and commit only to an update that has been agreed.

Choose your pace

Quick practice: spend 15 minutes reading one conversation and rehearsing one scenario. Full lesson: allow 45-60 minutes for language work, three conversations, two speaking scenarios, and feedback.

Level support

B1 learners can use the frames and a partner. B2 learners can try a response before reading the model. C1 learners can add the harder follow-up and reduce preparation time. These are teaching suggestions, not certified CEFR level ratings.

After practicing: open the AI prompt workshop to choose a vocabulary, grammar, role-play, or new-dialogue activity.

A 60-minute lesson that works

Prepare the case and the learner pages. Keep the model response hidden until learners have attempted their own. Choose two field terms that are important to understanding the case.

0-5 minutes | Activate experience

Ask learners to name a comparable situation without sharing confidential details. Introduce the communication goal.

5-12 minutes | Read and sort facts

Read the case. Learners identify confirmed facts, unknowns, the audience, and the immediate communication need.

12-22 minutes | Notice and practice language

Explain the workshop pattern. Work through the vocabulary and editing example, then discuss both language checks and the reasons behind the choices.

22-35 minutes | 05 • Conversations

Read one ten-turn conversation aloud, notice a useful expression, then compare the other two. Identify each speaker's purpose and how the exchange ends.

35-47 minutes | 06 • Say it

Rehearse both speaking scenarios from the workbook. Give two minutes of preparation, switch roles, and use the success checks. Add the complication only after a successful first round.

47-55 minutes | Compare and revise

Reveal the model. Discuss alternative acceptable wording. Give one meaning correction and one language correction, then repeat.

55-60 minutes | Retrieve and transfer

Close the materials. Learners recall two expressions and identify one communication habit to use in another situation.

Adapt the session

For 45 minutes, study one complete conversation and rehearse one scenario; assign the others for oral follow-up. For 90 minutes, rehearse all three conversations and give both scenarios a second round. More time should produce more meaningful practice, not longer explanations.

LESSON 01 / FACILITATOR NOTES

Confirming the visit and the care plan

Name the mismatch between the assignment and the client's expectation without blaming either person.

CASE

You arrive at 9:00 for a two-hour visit. Your assignment lists breakfast and light household tasks. The client expects transport to a 10:00 appointment, but transport is not listed and you have no authorization. The coordinator is available by phone.

Teach this move

A care plan records agreed support; a client's immediate request may differ. Acknowledge the need, identify what your current assignment actually says, and ask the person who can clarify it. An omission is not proof that nobody arranged support.

Vocabulary to check: care plan, assigned visit, scope of duties, client preference.

Model response

Your appointment is at ten, and I understand why you expected a ride. My assignment today lists breakfast and light household tasks, but it does not include transport. May I contact the coordinator now to check what was arranged? I cannot confirm a ride until we have that information.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revised statement describes the information you have without claiming to know every arrangement.

Second-round challenge

The coordinator says a separate transport service may have been contacted but needs five minutes to confirm. Explain the uncertainty and agree what to do while waiting.

Language-check answers

1. A - assigned visit Correct. An assigned visit is the client support allocated to this caregiver.
2. A - have not confirmed that Correct. This accurately marks what remains unknown.

05 • Conversations

1. The appointment is not on my assignment - A caregiver arrives for the fictional 9:00 visit and checks an unexpected transport request.
2. A changed start time - A caregiver and coordinator reconcile two different visit times before a planned home visit.
3. Whose preference is it? - A client explains how they want ordinary household support organized during the visit.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 • Say it: second scenario | A request from a visiting relative

During a two-hour visit, a relative asks you to clean the garage. Your assignment covers breakfast and light tasks in the living area. The client wants the remaining time used for laundry. No garage work is authorized.

Role A: Caregiver: acknowledge the relative, explain the assigned duties, and check the client's preference.

Role B: Relative: you thought any household cleaning was included. You can contact the coordinator about future services.

Possible opening: Could you clean out the garage before you leave today?

Success checks

- Identify the actual assignment without judging the relative.
- Keep the client's stated preference in the discussion.
- Refer any new service request to the coordinator without accepting a private arrangement.

Add a complication: The relative offers extra cash. Your employer requires service changes to go through the coordinator, not private arrangements.

LESSON 02 / FACILITATOR NOTES

Consent, choice, and respectful assistance

Make consent and preferences explicit while explaining a real time limit.

CASE

The client declines help arranging clothes before a visitor arrives. The visit has thirty minutes left. Choosing clothes is part of the agreed support, but the client prefers ten quiet minutes first. There is no immediate safety concern.

Teach this move

A polite question is not meaningful choice if a refusal is immediately ignored. State available options neutrally, allow an answer, and separate the person's preference from what the schedule allows.

Vocabulary to check: consent, personal space, reassurance, refusal.

Model response

You would like some quiet time before choosing clothes. We have thirty minutes left in today's visit. Would you prefer ten minutes alone and then help with the clothes, or would you rather leave that task today? Either way, I will respect your choice and confirm what support you want next.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The time limit is clear without making assistance sound compulsory.

Second-round challenge

The client chooses to leave the task today. Accept the decision and agree which remaining support, if any, they want.

Language-check answers

1. C - prefer Correct. 'Prefer ten quiet minutes' directly asks about the client's choice.
2. D - respect your decision Correct. This recognizes refusal while keeping other support available.

05 • Conversations

1. Ten minutes before we start - A client requests quiet time during a scheduled visit with thirty minutes remaining.

2. Let me finish the sentence - A caregiver repairs an overly hurried offer of help with a household task.
3. The family member answers first - A caregiver makes room for the client's own preference during a relative's visit.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 · Say it: second scenario | An unwanted photograph

A relative wants to photograph the caregiver helping the client choose clothes. The client says they do not want a photograph. Your employer requires consent and approval for work-related images. No image has been taken.

Role A: Caregiver: support the client's refusal and refer questions about photographs to the coordinator.

Role B: Relative: you wanted a family update, not a public post. You can describe the visit without taking a photograph.

Possible opening: Can you both smile for a quick photo to send the family?

Success checks

- Acknowledge the relative's purpose without treating it as consent.
- Restate that the client does not agree to a photograph.
- Offer an appropriate non-photographic way to give a general update.

Add a complication: The relative says the image is only for a private group. The client's refusal has not changed.

LESSON 03 / FACILITATOR NOTES

Reporting a change without diagnosing

Report a change using a source, time, and clear limit on your knowledge.

CASE

During a fictional routine visit, the client says, 'I feel more tired than yesterday.' You notice they stop twice while describing breakfast, which is unusual for your visits. You are not qualified to diagnose. No emergency signs are specified; use the local procedure for reporting changes and urgent concerns.

Teach this move

An observation is something you directly noticed; a reported symptom is the person's account. A baseline is their usual pattern, not a diagnosis. Do not convert one unfamiliar behavior into a clinical conclusion or delay urgent action to finish a language exercise.

Vocabulary to check: baseline, observation, reported symptom, escalation.

Model response

The client told me, 'I feel more tired than yesterday.' During our conversation, they paused twice before finishing their account of breakfast. That differs from my usual visits, but I cannot identify the cause. I am reporting the change through our agreed process and need the authorized clinician's direction on the next step.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revision separates the client's statement from the caregiver's observation and removes an unsupported diagnosis.

Second-round challenge

The supervisor asks whether the client is 'just tired.' Restate the facts and your knowledge limit without agreeing to an unsupported explanation.

Language-check answers

1. A - reported symptom Correct. The phrase identifies the statement as the client's own account.
2. A - cannot determine the cause Correct. This states a knowledge limit while preserving useful observations.

05 · Conversations

1. What did you actually notice? - A caregiver gives a factual phone report to a supervisor using the fictional case details.
2. A quieter morning - A caregiver checks the meaning of a client's unusual silence without attributing a cause.
3. A secondhand description - A coordinator helps a caregiver distinguish a relative's comment from a direct observation.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 · Say it: second scenario | An incomplete overnight report

A relative says the client 'wasn't herself last night' but gives no specific behavior. You have just arrived and have not yet observed a change. The supervisor is reachable. No emergency signs are supplied.

Role A: Caregiver: ask for observable details and timing, identify the source, and follow the agreed reporting process.

Role B: Relative: clarify that the client spoke less than usual at dinner at 7:00. You do not know why.

Possible opening: She wasn't herself last night. Can you tell the nurse something is wrong?

Success checks

- Ask what was seen or heard and when.
- Separate the relative's report from your own observations.
- Decline to diagnose and identify the appropriate reporting route.

Add a complication: The relative asks you to name a likely condition. You are not qualified to diagnose.

LESSON 04 / FACILITATOR NOTES

Boundaries around medicines and clinical questions

Respond helpfully to a request that exceeds the caregiver's authorized role.

CASE

Your fictional assignment permits reminders only, not administering medicines or recommending doses. The client asks whether to double a missed dose. You do not know the medicine details. The care plan identifies a nurse contact for medication questions.

Teach this move

A boundary is clearest when it names the specific decision you cannot make and the next person who can help. Do not suggest a dose, interpret a prescription, or let a relative's confidence substitute for authorized guidance.

Vocabulary to check: medication reminder, authorized task, clinical advice, referral.

Model response

I understand you want to know what to do about the missed dose. My role permits reminders, but I cannot recommend a dose or tell you to double it. Let's contact the nurse using the number in your care plan. I can help communicate your question without making the medication decision myself.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The second response keeps the boundary specific and provides an appropriate route to help.

Second-round challenge

The client says a neighbor gave an answer already. Explain why you still cannot endorse that advice and offer to help contact the appropriate professional.

Language-check answers

1. B - clinical advice Correct. A dosing question requires an appropriately qualified professional.
2. B - stay within my authorized role Correct. Another person's confidence does not change the assignment's limits.

05 • Conversations

1. Please tell me which dose - The caregiver responds to the fictional missed-dose request without offering medical advice.
2. A relative's new instruction - A caregiver discusses an unverified change to an authorized task with the coordinator.
3. The neighbor said it was fine - A client asks the caregiver to endorse advice received from a neighbor.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 • Say it: second scenario | An unreadable label

The client asks you to interpret a partly unreadable medicine label. Your assignment allows reminders only. The client has the pharmacy contact details, and your agency has an approved route for medication questions. No dosing decision has been made.

Role A: Caregiver: explain that you cannot fill in missing instructions from memory and help use the approved contact route.

Role B: Client: you believe you remember the wording but are not certain. You agree to ask an authorized professional.

Possible opening: The print has rubbed off. Can you tell me what this instruction probably says?

Success checks

- Identify the information as unreadable or unverified.
- Do not interpret or invent the missing instruction.
- Offer a concrete route for obtaining authorized clarification.

Add a complication: The client finds an old label from a previous supply and asks whether it must be the same.

LESSON 05 / FACILITATOR NOTES

Meals, preferences, and unresolved restrictions

Separate an available approved option from a requested but unverified substitution.

CASE

The plan lists two approved breakfast options. The client dislikes the first and asks for a third food that is not on the list. You do not know whether it meets the documented restrictions. The second approved option is available. The coordinator can clarify the third option.

Teach this move

A preference describes what someone wants; a restriction limits what is appropriate under their plan. The caregiver should not invent a clinical reason for a restriction or promise that a substitute is suitable before it has been checked.

Vocabulary to check: dietary restriction, substitution, meal preference, intake record.

Model response

You don't want the first breakfast option today. The second option in your plan is available, or I can ask the coordinator about the food you requested. I don't know whether that substitution fits your documented restrictions, so I should not describe it as approved. Which of those next steps would you prefer?

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revision reports the actual information gap without judging the client's choice or inventing a medical explanation.

Second-round challenge

The client asks why the restriction exists. You do not know the clinical reason; acknowledge the question and refer it without making one up.

Language-check answers

1. A - preference Correct. The client is expressing what they prefer among permitted options.
2. A - what I actually observed Correct. A factual record distinguishes observed quantity from assumptions.

05 · Conversations

1. A third breakfast option - Fictional care plan: today's listed options are porridge or toast. Pancakes have not been reviewed as a substitute; this exchange does not assess dietary suitability.
2. Half a portion is not no breakfast - Two caregivers clarify the meaning of a meal record during a handoff.
3. A shopping substitution needs checking - A caregiver checks a proposed grocery substitute with a coordinator rather than interpreting dietary instructions.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 · Say it: second scenario | The relative's favorite recipe

A relative brings ingredients for a meal that is not on the client's approved list. The client would like to try it. You cannot confirm whether it fits a documented restriction. A listed alternative is available now.

Role A: Caregiver: distinguish a welcome offer from an approved meal and propose an authorized check.

Role B: Relative: you want to help and did not know there was a restriction. You can provide the ingredient information.

Possible opening: I brought everything for his favorite meal. Can you prepare it today?

Success checks

- Acknowledge the offer without endorsing the proposed meal.
- Explain exactly what remains unverified.
- Offer the existing approved choice and the appropriate clarification route.

Add a complication: The relative says removing one ingredient should solve the issue. No authorized person has confirmed that.

LESSON 06 / FACILITATOR NOTES

Privacy and family communication

Explain an identity or permission check and provide a legitimate route for the caller's request.

CASE

An unfamiliar caller says she is the client's niece and asks what happened during the visit. The contact record is unavailable to you at that moment. You cannot verify her identity or permission. Your coordinator can handle the request through the approved process.

Teach this move

Knowing a client's name or address is not proof of authorization. Family relationships do not tell you exactly what may be shared. Use the organization's verification procedure, avoid confirming private facts unnecessarily, and do not promise access you cannot authorize.

Vocabulary to check: confidentiality, authorized contact, identity verification, approved channel.

Model response

I understand you are concerned about your aunt. Before discussing visit details, I need to verify the contact arrangement through our approved process. I cannot confirm private information on this call. Please contact the coordinator through the agency's usual number, or I can pass on your request without sharing the client's details.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revised sentence does not treat an unverified relationship as permission to disclose information.

Second-round challenge

The caller says the check is insulting. Acknowledge the frustration and explain that the same process protects every client's privacy.

Language-check answers

1. D - confirmation of permission to receive the information Correct. Identity and permission are separate checks; permission remains unresolved here.
2. A - approved channel Correct. The employer's approved channel is the appropriate route.

05 • Conversations

1. An unfamiliar caller - A caregiver handles a request for private visit information from an unverified person.
2. The conversation in the hallway - A caregiver moves a potentially private discussion away from a shared building corridor.
3. A convenient but unapproved message - Two caregivers discuss how to share a handoff without using a personal messaging group.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 • Say it: second scenario | A family group invitation

A relative invites you into a family messaging group to send daily visit updates. The agency has not approved that group, and the client's sharing preferences have not been checked. The coordinator can explain approved communication options.

Role A: Caregiver: decline the unapproved channel and offer the established route for updates.

Role B: Relative: you want everyone to receive the same information. You can ask the coordinator to clarify authorized contacts.

Possible opening: I'll add you to our family chat so you can post a quick update after every visit.

Success checks

- Explain that removing a name does not establish permission or channel approval.
- Avoid sharing visit details while discussing the request.
- Direct the family to the coordinator for a verified arrangement.

Add a complication: The relative offers to remove the client's name from the messages, but the group members would still know who the updates concern.

LESSON 07 / FACILITATOR NOTES

A handoff the next person can use

Separate completed work, unresolved work, and responsibility for the next step.

CASE

At 11:00 you finish a fictional visit. Breakfast support and laundry are complete. A transport query for tomorrow's 10:00 appointment is still pending with the coordinator. The incoming caregiver starts at 15:00. The coordinator, not the incoming caregiver, is arranging transport.

Teach this move

A handoff is not just a list of events. The receiving person needs to know what matters for their visit, which facts remain uncertain, and who is acting. A read-back catches misunderstandings before 'requested' becomes 'confirmed'.

Vocabulary to check: handoff, pending action, read-back, follow-up owner.

Model response

Breakfast support and laundry are complete. Transport for tomorrow's ten o'clock appointment is still unconfirmed; the coordinator owns that follow-up. Your visit starts at three today. Please check the coordinator's update through the approved channel, and do not describe the ride as booked until confirmation arrives. Could you read back the pending item and who is handling it?

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revised handoff names the pending action, the owner, and the limit on certainty.

Second-round challenge

The incoming caregiver assumes that checking the update means personally booking another ride. Correct the misunderstanding before a duplicate request is made.

Language-check answers

1. D - pending Correct. Pending means the arrangement is awaiting an outcome.
2. A - follow-up owner Correct. This identifies responsibility for the unresolved action.

05 · Conversations

1. Requested is not booked - Outgoing and incoming caregivers confirm the status of tomorrow's transport.
2. Which appointment did you mean? - A read-back exposes two different appointment dates during a routine handoff.
3. An unread update - A caregiver checks whether a time-sensitive handoff has actually reached its recipient.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 · Say it: second scenario | An action with two assumed owners

The outgoing caregiver thinks the coordinator ordered replacement household supplies. The coordinator thinks the incoming caregiver will ask the client what is needed first. No order has been placed. The incoming visit starts at 15:00.

Role A: Outgoing caregiver: clarify the actual status and obtain an explicit owner and next step.

Role B: Coordinator: confirm that you will arrange the approved order after the incoming caregiver checks the list with the client.

Possible opening: Before I hand over, can we confirm whether the supply order has actually been placed?

Success checks

- Keep an assumption separate from confirmed status.
- Name the person who checks the list and the person who places the order.
- Ask for a read-back of the sequence and timing.

Add a complication: A colleague says, 'It's probably already done.' No confirmation or order record is available.

LESSON 08 / FACILITATOR NOTES

Delays, complaints, and reliable follow-up

Acknowledge the disruption, label an estimate, and commit only to an update that has been agreed.

CASE

Your fictional 9:00 visit will be delayed. At 8:50, dispatch estimates arrival between 9:30 and 9:45 but cannot guarantee it. The client had planned breakfast support at 9:00. Dispatch is checking an approved backup arrangement and will update the client by 9:10.

Teach this move

People need practical information during delays. Separate the estimated arrival from the promised next contact, and explain the escalation route if the update is missed. Apologizing should not lead to an invented guarantee or an unauthorized change to support.

Vocabulary to check: visit window, service disruption, acknowledgment, contingency plan.

Model response

I'm sorry this delay affects your planned breakfast support. Dispatch currently estimates arrival between nine thirty and nine forty-five, but that is not a guaranteed time. They are checking the approved backup arrangement and will update you by nine ten. I will not describe the backup as confirmed until dispatch verifies it.

Listen for, then coach

Ask learners to point to the case fact behind each important statement. Accept different wording when it preserves meaning, fits the listener, and does not invent an outcome. The revised message distinguishes a confirmed communication commitment from an uncertain arrival estimate.

Second-round challenge

At 9:10 no update has arrived. Use the approved follow-up route and describe the missed update accurately without assuming a backup was cancelled.

Language-check answers

1. B - estimate Correct. An estimate describes a current expectation, not a guarantee.
2. B - give you a status update Correct. A status update can report progress without promising that arrangements are complete.

05 • Conversations

1. A delay before breakfast support - A caregiver informs the client of a delay before the original visit window.
2. The update was missed - A caregiver follows up with dispatch after an agreed callback time passes.
3. A complaint after the visit - A client explains why a small delay felt more serious than the worker expected.

Use the complete ten-turn scripts in the learner workbook and conversation lab. Read with roles, identify the decision or open question, then replay without reading.

06 • Say it: second scenario | A worker cannot access the building

You arrive within the visit window, but the authorized entry method does not work and the client does not answer through the normal contact route. You do not know why. Your agency has a no-access procedure and a coordinator on duty.

Role A: Caregiver: report the access problem accurately and follow the approved procedure without inventing a reason or an unsafe entry method.

Role B: Coordinator: ask which approved contact steps were attempted and when; you will direct the next authorized action.

Possible opening: I'm at the scheduled address, but I cannot gain access through the approved entry method.

Success checks

- Give the time, location context, and actual access issue through the approved channel.
- Distinguish no response from a conclusion about the client's location or condition.
- Follow the agency's no-access and urgent-concern procedures without improvising entry.

Add a complication: A neighbor offers an unverified explanation and suggests an unofficial way inside. You must not treat either as authorization.

Give feedback that helps

Score each criterion 0, 1, or 2. This is a practice rubric, not a professional qualification or CEFR test. Do not award or remove points for a particular accent.

Meaning and accuracy

Keeps the case facts accurate; clearly separates confirmed and unknown information.

Clarity and language

Uses understandable sentences and explains specialist terms when the listener needs it.

Interaction and tone

Responds to the other person's question, checks understanding, and uses an appropriate tone.

Task completion

Makes the requested action or unresolved question clear without inventing authority or facts.

Use the same scale for each criterion

0 - Not yet: The reader or listener cannot yet recover the intended meaning or action.

1 - With support: The message works after a prompt, clarification, or revision.

2 - Independently: The message is clear and accurate without a prompt.

Feedback sequence

First, identify a successful phrase. Next, identify one point where meaning or accuracy needs work. Offer one useful language correction, allow a repeat, and compare the two attempts. A total out of eight describes this performance only; do not translate it into a proficiency level.

Final challenge

Choose a case not rehearsed today. The learner gives a one-minute response, answers two follow-up questions, and switches roles and repeats the exchange. Add the lesson's second-round challenge. Compare the result with an earlier attempt using the four criteria.

Use the language responsibly

Fictional workplace-language practice, not clinical training. Duties vary by qualification, care plan, employer, and location. Obtain consent, follow approved procedures, protect private information, and refer clinical or emergency decisions to authorized professionals.

Meaning before performance

Use specialist terms only when they help the listener. Explain unfamiliar words, preserve uncertainty, and ask when an instruction or responsibility is unclear. The model responses illustrate language; they do not authorize professional actions.

Sources and further reading

The cases, workshops, and explanations are original teaching material. These references provide language frameworks and selected professional context. References were checked in September 2026; consult current local guidance for actual work.

Digital.gov: plain-language guidance

<https://digital.gov/guides/plain-language>

Council of Europe: CEFR descriptors

<https://www.coe.int/en/web/common-european-framework-reference-languages/cefr-descriptors>

BLS: Home Health and Personal Care Aides

<https://www.bls.gov/ooh/healthcare/home-health-aides-and-personal-care-aides.htm>

CDC: Plain Language Materials

<https://www.cdc.gov/health-literacy/php/develop-materials/plain-language.html>

BLS: Personal Care Aides Occupational Requirements

<https://www.bls.gov/ors/factsheet/pdf/personal-care-aides.pdf>

Your next step

Choose one expression you can use in a genuine work situation. Rehearse it with fictional details, then adapt the wording to your role and audience.